Benevolent House

Licensed Therapist Orientation Packet

Purpose: the policies and TST-FC reference pages this orientation is built on.

What is in this packet

1. T3C Service Delivery Addendum, Policy T3C-2: Treatment Model Integration

How TST-FC shapes service planning, therapy, behavior support, and family connection.

2. Policy T3C-5: Service Planning (Sections D and F)

Arranging therapy, written justification of frequency, and safety and crisis plans.

3. Policy T3C-6: Quality Assurance and Continued Stay Guidelines

Continued stay criteria and the 90-day written confirmation.

4. Staffing Functions Packet: Licensed Therapist scope of work and policies T3C-S2, T3C-S4, T3C-S5

Your role, oversight of the agreement, capacity review, and 24/7 crisis response.

5. TST-FC reference pages

Survival-in-the-moment states, the trauma system, the four R's, the Moment-by-Moment Assessment, the priority problem, predictability, the Managing Emotions Guide, and emotional hotspots.

Pages 1 and 10 of the addendum excerpt also show the start of the neighboring policy. You may skip that text.

Benevolent House

T3C Service Delivery Addendum to the Policy and Operations Manual

Purpose: This addendum sets the policies and procedures Benevolent House follows to deliver services under the Texas Child-Centered Care (T3C) System.

Service Packages: T3C Basic Foster Family Home Support Services; Mental & Behavioral Health Support Services; Kinship Caregiver Support Services Add-On

Version 3, revised October 23, 2026. Incorporates Version 2 (DFPS enhancement response, October 15, 2026).

T3C-1 Scope and Authority

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To state which services this addendum covers and how it relates to the Benevolent House Policy and Operations Manual.

POLICY

Benevolent House delivers T3C services according to the T3C System Blueprint published by the Texas Department of Family and Protective Services, the Minimum Standards for Child-Placing Agencies, and its contracts with DFPS and each Single Source Continuum Contractor (SSCC). This addendum applies to every child, youth, and young adult served under a T3C Service Package or Add-On Service for which Benevolent House holds a Credential.

PROCEDURE

1. The Policy and Operations Manual remains in effect. This addendum adds the requirements that are specific to T3C.
2. Where this addendum and the manual differ for a child served under T3C, this addendum controls.
3. The Executive Director reviews this addendum each time DFPS publishes a new edition of the T3C System Blueprint and brings needed changes to the Board of Directors.
4. Staff and caregivers are trained on this addendum before Benevolent House begins operating under an Active Full Credential and whenever it changes.

T3C-2 Treatment Model Integration

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026 **Revised:** October 23, 2026

PURPOSE

To make sure Trauma Systems Therapy for Foster Care (TST-FC) is practiced throughout Benevolent House and is the basis for policy, procedure, and practice in each Service Package and Add-On Service.

POLICY

TST-FC is the Treatment Model for all children served by Benevolent House under T3C. Every staff member, caregiver, and contracted provider who serves these children is trained in the model, practices it, and stays current through ongoing training: four one-hour TST-FC sessions a year, one each calendar quarter, delivered live, by video conference, through the Benevolent House online training portal, or one-to-one at a home visit, as set out in the TST-FC Ongoing Training Curriculum, with the sessions required of each contracted role stated in the T3C Training Curriculum and Plan. Each child's Service Plan is written in the terms of the model. The model is applied to the custom needs of each Service Package and Add-On Service as set out in Section G.

PROCEDURE

A. Admission and matching

1. Admissions staff review the Application for Placement, the child's trauma history and known triggers, and the CANS 3.0 Assessment when one has been completed.
2. Staff select a caregiver who is credentialed for the child's Service Package, has completed TST-FC training, and can provide the level of predictability and supervision the child needs.
3. Before or at placement, the case manager tells the caregiver the child's known triggers, early warning signs, and what has helped the child calm in the past.

B. Service planning

1. Each Service Plan includes, in TST-FC terms: the child's priority problem; known triggers; signals of safety; regulation strategies for each of the four stages of behavior; and goals for both the child and the caregiving environment.
2. The child takes part in service planning when appropriate for the child's age and development, and the child's own words are used for triggers and calming strategies where possible.
3. Results of the most recent CANS 3.0 Assessment are used to set and adjust the type, frequency, and duration of services.
4. Each Service Plan review documents progress toward each goal and any change to the Managing Emotions Guide.

C. Case management and caregiver coaching

1. At each monthly visit the case manager asks about episodes since the last visit, uses the moment-by-moment assessment to look at what happened before, during, and after, and coaches the caregiver on the next step.
2. The case manager records the coaching provided and the caregiver's use of the model in the visit note.
3. The quarterly foster home evaluation includes a review of the caregiver's understanding and use of TST-FC.

D. Therapeutic and clinical services

1. Therapy is arranged through STAR Health at the frequency the Service Planning team determines, and the Service Plan states the assessed need and frequency.
2. The case manager shares the child's priority problem and Managing Emotions Guide with the child's therapist and invites the therapist to each Service Plan meeting.
3. For the Mental & Behavioral Health Support Services Package, the Treatment Director and the Licensed Therapist review treatment and service planning as described in Section G.

E. Behavior support and discipline

1. Caregivers use predictability, pre-teaching, staying neutral, time-in, and restorative discipline.
2. Punishment that shames, isolates, or repeats a child's experience of harm is not used.
3. After any serious incident or emergency behavior intervention, the case manager completes a moment-by-moment review with the caregiver and updates the Managing Emotions Guide when the review shows a new trigger or a strategy that did not work.

F. Recreation, Normalcy, and family connection

1. Caregivers plan Normalcy activities that build regulation, resilience, and connection, and plan ahead for times that are hard for the child.
2. Caregivers and case managers support contact with parents, siblings, relatives, and other supportive persons when it is safe and approved by the DFPS or SSCC caseworker.

G. Programming by Service Package

Each Service Package and Add-On Service serves a population with custom needs. The table states those needs, how the model is designed to meet them, and the programming specific to the package. The Treatment Model Narrative, Section 4, describes each in full.

Service Package	Custom needs, how the model is designed for them, and programming specific to the package
T3C Basic Foster Family Home Support Services	**Custom needs.** Children removed from home because of abuse or neglect whose needs are relational and developmental rather than clinical: a predictable home, adults whose reactions can be anticipated, help with ordinary moments their history has made unsafe, continuity in school and community, and protection of ties to family and culture. **How the model is designed for them.** Predictability and routines are the first intervention. The Managing Emotions Guide is the working document. Pre-teaching and signals of safety are built into everyday parenting. The case manager coaches the caregiver at each monthly visit with the Moment-by-Moment Assessment. TST-FC is applied in everyday parenting: routines, transitions, pre-teaching, and signals of safety. A Managing Emotions Guide is built with each child old enough to take part. Service Plan review at least every six months. CANS 3.0 Assessment annually for children over age 3. The Program Director reviews goals and services with each six-month review. **Protection from re-traumatization.** Restorative discipline only; no shame, isolation, withholding of food or family contact, or consequence that repeats a child's experience of harm. No processing of an episode while the child is in a survival state. Known triggers shared with the caregiver before or at placement. Family and sibling connections protected. When a move is unavoidable, the Managing Emotions Guide and strategies go with the child.
Mental & Behavioral Health Support Services	**Custom needs.** Children who have, or are being evaluated for, an emotional, conduct, or behavioral disorder: more frequent, intense, and dangerous episodes; therapy and often medication that must be coordinated with the home; higher risk of crisis, serious incident, hospitalization, and disruption; caregivers at higher risk of burnout. They need the home and the clinic working from one plan, adults who stay regulated through a long episode, and a path back to the Basic package in the same home. **How the model is designed for them.** All Basic programming, plus a written safety and crisis plan tied to the four stages, one plan across home and clinic through the Treatment Director and Licensed Therapist, review every 90 days, crisis response that uses the child's own guide, more coaching for the caregiver, and step-down to Basic in the same home. All Basic programming applies. A written safety and crisis plan is tied to the child's four stages of behavior and is shared with the caregiver, therapist, and on-call staff. Case manager level or above staff respond to any crisis 24 hours a day, 7 days a week. Caregivers take part in therapy with the child as needed, attend multiple meetings, and respond immediately to the child's mental and behavioral health needs. The Licensed Therapist provides oversight and input on treatment and service planning, including the frequency of therapy. Service Plan review and CANS 3.0 Assessment at least every 90 days. The Program Director and Treatment Director provide written continued stay confirmation with each review. An Aftercare Services plan is prepared at discharge, with follow-up for six months. **Protection from re-traumatization.** Everything in Basic, plus: emergency behavior intervention is a last resort within the caregiver's Minimum Standards training and is followed every time by a Moment-by-Moment review and an update to the guide and crisis plan; in a survival moment the other children are moved, not the child in crisis; psychotropic medication is monitored with the medical consenter and the child's input; hospitalization and placement change are planned whenever possible and the child's guide travels with the child; caregiver exhaustion is treated as a clinical risk to the child and met with paid Intermittent Alternate Care and added support.

2. Admissions staff record every referral in the T3C Referral, Admission, and Discharge Tracker on the day it is received, with the referral source and the Service Package requested.
3. Admissions staff review the referral against the guidelines in A and B and identify a caregiver as described in Policy T3C-2, Section A.
4. When a child may fit more than one Service Package, the primary Service Package is determined by agreement between Benevolent House and the SSCC or DFPS.
5. Admissions staff give the SSCC or DFPS an answer on every referral. When a child is not admitted, the reason is stated in the response and recorded in the tracker.
6. The Program Director approves each admission and consults the Treatment Director on admissions under the Mental & Behavioral Health Support Services Package. For a child on the Program Director's own caseload, the Executive Director approves.
7. The admission decision, the Service Package, and the basis for the decision are documented in the child's case record.

T3C-5 CANS 3.0 Assessment, Service Planning, and Arranging Services

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026 **Revised:** October 15, 2026

PURPOSE

To set the day-to-day procedures for assessment, service planning, therapy and other services, and care and supervision plans for children served under T3C.

POLICY

Each child has a customized Service Plan that is informed by the child, built with the Service Planning team, guided by the most recent CANS 3.0 Assessment, and written in the terms of the Treatment Model. Benevolent House arranges every service the plan calls for and documents the result.

PROCEDURE

A. Schedule

Requirement	Basic	Mental & Behavioral Health
CANS 3.0 Assessment, children over age 3, youth, and young adults	At least once every 12 months.	At least once every 90 days.
Service Plan review	At least once every six months, and more often when the child's needs or permanency plan call for it.	At least once every 90 days, and more often when the child's needs or permanency plan call for it.
Therapy	Individual and family therapy at the frequency set by the Service Planning team.	Individual, family, and group therapy at the frequency set by the Service Planning team and the Licensed Therapist.
Crisis response	On-call staff are available to caregivers 24 hours a day, 7 days a week.	Case manager level or above staff respond to any crisis in person, by phone, or by video conference 24 hours a day, 7 days a week.

A child receiving the Kinship Caregiver Support Services Add-On follows the schedule of the child's primary Service Package.

B. CANS 3.0 Assessment

1. The case manager tracks the due date of each child's CANS 3.0 Assessment and makes sure it is completed by a certified CANS assessor within the timeframe in Section A.
2. The case manager obtains the results and files them in the child's case record.

3. At each Service Plan review, the Service Planning team reviews the most recent results and uses them to set or adjust the type, frequency, and duration of services. The Service Plan review states which results were considered and what changed because of them.
4. The Case Manager Supervisor confirms at each review that the assessment is current.

C. Service Plan

1. The Service Plan includes customized goals and the planned services and supports that will help the child reach each goal, along with the Treatment Model content required by Policy T3C-2, Section B.
2. The child, when appropriate, and each youth and young adult help shape the plan. Parents, relatives, and supportive persons are included as described in Policy T3C-8.
3. Each Service Plan review documents the progress made toward each goal.

D. Arranging therapy and other services

1. Therapy and other Medicaid-covered services are arranged through STAR Health with providers who are credentialed and contracted with STAR Health. The case manager sends authorization requests to STAR Health as needed.
2. The Service Plan includes a written justification of the child's assessed need for therapy, including the frequency.
3. For the Mental & Behavioral Health Support Services Package, therapy is provided by a Licensed Therapist with experience treating children with emotional, behavioral, and conduct disorders, unless the Service Planning team determines that a different type of therapist is needed. Foster family home caregivers participate in therapy with the child as needed. Caregivers must have the ability to attend multiple meetings and to respond immediately based on the child's mental and behavioral health needs. The case manager confirms this ability when the home is credentialed and re-credentialed for this Service Package under Policy T3C-10 and Attachment D, and reviews it at each 90-day Service Plan review.
4. In addition to holding the Credential for the Mental & Behavioral Health Support Services Package, Benevolent House and each foster family home that serves a child under that package also hold the Credential for the T3C Basic Foster Family Home Support Services Package. This allows the child to transition to, and receive, the T3C Basic Foster Family Home Support Services Package in the same foster family home when the SSCC or DFPS, in collaboration with Benevolent House and as informed by the Quality Assurance and Continued Stay Guidelines in Policy T3C-6, determines that the child has successfully completed, and no longer requires, the therapeutic and recovery services of the Mental & Behavioral Health Support Services Package.
5. The case manager and caregiver coordinate care with the child's medical consenter and take part in STAR Health Service Coordination when the child is eligible.
6. The case manager acts as the child's education liaison: takes part in school enrollment, requests educational testing when needed, takes part in Admission, Review, and Dismissal and Section 504 meetings as needed, and confirms that accommodations and supports are in place.
7. Caregivers provide transportation, logistical support, and record keeping for services as required by court orders and the Service Plan.

E. Documenting services and access to services

1. In collaboration with the medical consenter, the case manager documents in the case record all services the child receives through STAR Health, HHSC Behavioral Health Services, Early Childhood Intervention, the school, and any other county, community, or state agency.
2. When a service called for in the Service Plan is not readily available, or the child is found ineligible, the case manager documents the date of the request, application, or referral; the type of service requested; the status of the request; the reason given for any denial; and the status of any appeal.
3. The case manager notifies the SSCC or DFPS caseworker within 3 business days of any difficulty obtaining a service or any denial of a service referral.
4. The case manager seeks community resources for any needed service that STAR Health does not cover.

5. When a service called for in the Service Plan is consistent with the child's Service Package and is not available through STAR Health or community resources, the case manager brings the need to the Program Director, and Benevolent House makes sure the service is delivered and covers its cost.

F. Care and supervision plans

1. The Service Plan states the supervision the child requires and any special care instructions.
2. Each child in the Mental & Behavioral Health Support Services Package has a written safety and crisis plan tied to the four stages of behavior. The plan is shared with the caregiver, the therapist, and on-call staff, and is reviewed at each 90-day review and after any serious incident.
3. Benevolent House foster family homes care for no more than six children. If HHSC Child Care Regulation grants a variance that allows a home to care for seven or more children, awake night supervision is provided in that home.
4. Caregivers support Normalcy activities that fit the child's age and Service Plan, including clothing, hygiene products, hair care, birthdays, holidays, and graduations.

T3C-6 Quality Assurance and Continued Stay Guidelines

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To make sure each child continues to receive the Service Package that fits the child's needs, and that every required review, written confirmation, and notification is completed.

POLICY

Benevolent House reviews each child's services on a set schedule. At each review, the responsible directors confirm in writing that the child continues to meet the criteria for the Service Package and is benefitting from the Treatment Model. A child whose needs have changed is moved to the package that fits.

PROCEDURE

A. Continued stay guidelines

A child continues in a Service Package when all of the following are true.

1. Services are being reviewed and adjusted based on the most recent CANS 3.0 Assessment and the Service Plan.
2. The primary reason the child met the Admission Guidelines continues to require ongoing services, or that reason has been replaced by other service needs that fit the Service Package and meet the Admission Guidelines.
3. The services continue to support the child's need for safety, improved well-being, and permanency, in accordance with the child and family Service Plans.
4. A less restrictive placement type is not appropriate to meet the child's needs.
5. For the Mental & Behavioral Health Support Services Package, the child's needs continue to call for a level of support that the Basic package does not provide.
6. Benevolent House and the foster family home continue to hold the Credential for the Service Package.

B. Review and written confirmation

Service Package	When	Who reviews and signs	What the written confirmation states
T3C Basic Foster Family Home Support Services	With each six-month Service Plan review	Program Director	The child continues to meet the criteria for the package and is benefitting from the Treatment Model.
Mental & Behavioral Health Support Services	With each 90-day Service Plan review	Program Director and Treatment Director	The child continues to meet the criteria for the package and is benefitting from the Treatment Model, and the Basic package is not appropriate to meet the child's needs.

1. The case manager gives the reviewing directors the most recent CANS 3.0 Assessment results, the Service Plan review, and the progress recorded for each goal.
2. The directors review the child's goals and services to make sure they align with the child's strengths, needs, and permanency plan, and apply the guidelines in Section A.
3. The directors complete and sign the Continued Stay Written Confirmation (Attachment B). For a child on the Program Director's own caseload, the Executive Director signs in the Program Director's place, as stated in Policy T3C-S1.
4. The signed confirmation is filed in the child's case record.
5. The case manager sends a copy to the SSCC or DFPS caseworker within 15 business days of the review and confirmation, and records the date sent on the form.

C. When a child no longer meets the guidelines

1. When the review shows that a child in the Mental & Behavioral Health Support Services Package has completed, and no longer requires, the therapeutic services of that package, the Program Director notifies the SSCC or DFPS caseworker. When the SSCC or DFPS agrees, the child moves to the Basic package and remains in the same foster family home.
2. When the review shows that a child needs a Service Package that Benevolent House does not hold, the Program Director notifies the SSCC or DFPS caseworker and takes part in planning the transition. The child's Managing Emotions Guide and records go with the child.
3. The change is recorded in the T3C Referral, Admission, and Discharge Tracker with the date and the reason.

D. Quality assurance

1. The Case Manager Supervisor tracks the due date of every Service Plan review, CANS 3.0 Assessment, and written confirmation.
2. On-time completion of reviews and confirmations is reviewed each quarter through the Continuous Quality Improvement process.

T3C-7 Anticipated Length of Service

Board Approved: October 2, 2026 **Effective Date:** October 2, 2026

PURPOSE

To state the length of service Benevolent House anticipates for each Service Package and how actual length of service is tracked.

POLICY

Length of service is individualized. It is based on the Treatment Model, the Admission Guidelines, the Continued Stay Guidelines, the child's CANS 3.0 Assessment, and the child's progress under the Service Plan. No child is discharged or moved to another Service Package because an anticipated length of service has been reached. That decision is made only through the Continued Stay Guidelines.

PROCEDURE

Service Package	Anticipated length of service	Basis
T3C Basic Foster Family Home Support Services	12 to 18 months	Services continue until the child reaches permanency through reunification, placement with a relative, adoption, or transition to adult living.
Mental & Behavioral Health Support Services	6 to 12 months	Reviewed every 90 days. When the child no longer requires the therapeutic services of this package, the child moves to the Basic package in the same foster family home.

Licensed Therapist: Scope of Work and Related Policies

Adapted from the Benevolent House T3C Staffing Functions Packet and the T3C Training Curriculum and Plan, Version 2

Licensed Therapist (scope of work)

Service Package	Mental & Behavioral Health Support Services
Assigned to	A contracted Licensed Therapist, under a written agreement. The Treatment Director oversees the agreement.
Reports to	Treatment Director
Responsibilities	Provides oversight and input on treatment and service planning for children in the package, including the frequency of therapy. Takes part in Service Plan reviews when invited. Any therapy the provider gives a child is arranged and billed through STAR Health when the provider is credentialed and contracted with STAR Health, separate from the agreement.
Qualifications	Licensed therapist with experience treating children with emotional, behavioral, and conduct disorders. Completes the Benevolent House TST-FC orientation before providing oversight of treatment and service planning, then Sessions 2 and 3 of the TST-FC Ongoing Training each year.

T3C-S2 Workload Review, item 6: Contracted therapist

The Treatment Director confirms each quarter that the contracted provider has the capacity to serve the children referred, and arranges another STAR Health provider when it does not. The Blueprint reference ratio for the Licensed Therapist is 1 to 14 children in the Mental & Behavioral Health package. Ratios may vary based on the Treatment Model and the complexity of the caseload.

T3C-S4 Oversight of Contracted Functions

A function provided under contract is held to the same requirements as a function performed by staff. Each contracted function is covered by a signed written agreement kept on file. The Treatment Director oversees the Licensed Therapist agreement, verifies each provider's license, and confirms the provider has completed the TST-FC orientation before the provider gives oversight of treatment and service planning. The overseeing staff member raises any concern with the contractor in writing and reports unresolved concerns to the Executive Director.

T3C-S5 Crisis Management and On-Call Response

Case manager level or above staff are available 24 hours a day, 7 days a week, to respond to any crisis involving a child in the Mental & Behavioral Health Support Services Package. Every caregiver is given the on-call number at placement. On-call staff respond by phone, by video conference, or in person, and have access to each child's safety and crisis plan and Managing Emotions Guide. The contracted Licensed Therapist is not part of the on-call rotation.

MODULE 1

Survival-in-the-Moment States

When coping fails, the whole system switches over to survival. Four things are affected at once.

1

Awareness

Of self and of the environment. The child may not fully register where they are or who is with them.

2

Feelings

Intense and fast. Rage, terror, panic. Or the opposite: numbness and shutdown.

3

Physical responses

Heart rate, breathing, muscle tension, digestion. The body is preparing to act.

4

Ability to cope

The skills the child has on a good day are simply not available in this state.

Three ways a body goes into survival

FIGHT

Becomes physically or verbally aggressive. Hits, screams, curses, throws.

FLIGHT

Flees the situation. Runs away, hides, leaves the room, goes to the nurse.

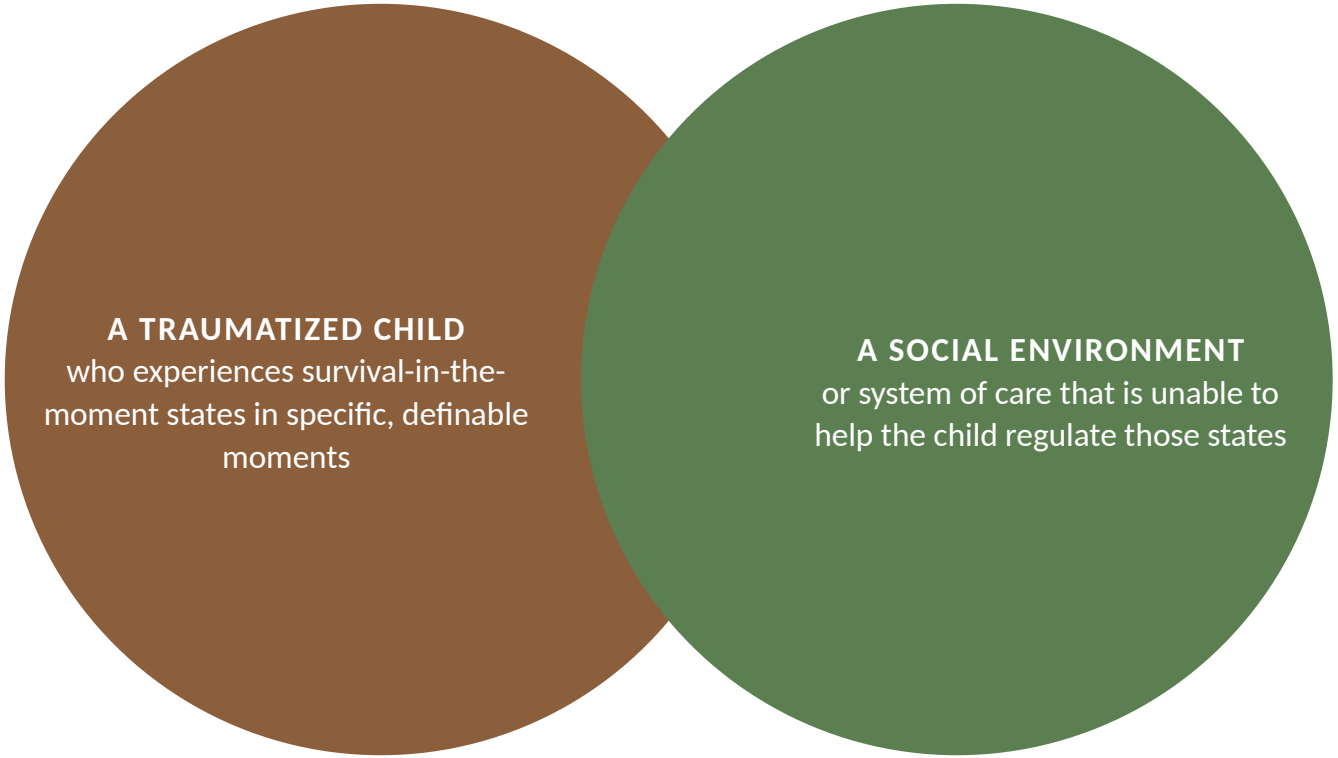
FREEZE

Shuts down physically and emotionally. Stops talking, no eye contact, goes blank.

MODULE 1

The Trauma System

Two halves, equally important. TST-FC works on both at once, which is what makes it a systems model.



SO YOU DO TWO THINGS AT ONCE

1 **Work from the inside out**
Build the child's own skills and resources so they can manage a survival response when it comes.

2 **Work from the outside in**
Change the environment so the reminders come less often and the support arrives faster.

Just asking what happened is not enough. Learn what is going on in the child's environment: in your home, at school, during time with family, in the neighborhood, and in their faith and cultural communities.

Small changes work the same way. Just as negativity in one part of a child's life spreads into the others, so do small positive changes. Children who have experienced trauma also have real strengths, and those strengths are highly adaptive.

MODULE 2

The Four Stages of Behavior: the 4 R's

How the child acts changes by stage, so what you do has to change by stage too.

REGULATING	REVVING	RE-EXPERIENCING	RECONSTITUTING
Being in control	*Getting upset*	*Losing control*	*Getting it back together*
CHILD BEHAVIOR Restful. Calm and engaged in their environment.	CHILD BEHAVIOR Vigilant. Triggered, and trying to manage it.	CHILD BEHAVIOR Fight, flight or freeze. Coping is overwhelmed.	CHILD BEHAVIOR Calming down. Beginning to manage and re-engage.
YOUR PRIORITY **Minimize triggers to prevent escalation.**	YOUR PRIORITY **Help your child regulate emotion. Calm, do not teach.**	YOUR PRIORITY **Keep everyone safe without re-traumatizing the child.**	YOUR PRIORITY **Support the return. Do not rush it.**

Watch for the 3 A's to tell you which stage you are in: AFFECT, what their face and body show. AWARENESS, what they are focused on. ACTION, what they are doing. When all three shift inside one episode, you are looking at traumatic stress rather than ordinary misbehavior.

MODULE 2

The Moment-by-Moment Assessment

One episode. Three windows. This is how you find the cat hair.

JUST BEFORE

- How did the child look emotionally?
- What were they doing?
- What appeared to trigger it?
- Who initiated the trigger?
- Where did it happen?

DURING

- What did the child appear to be feeling?
- What was the child doing? Describe the behavior, not the label.

AFTER

- What did the child appear to be feeling?
- What were they doing?
- How long to return to baseline, and what helped?
- If you could name one thing bothering this child, what would it be?

TWO RULES THAT MAKE IT WORK

1. Two separate episodes, minimum. One episode is an anecdote. Two is the beginning of a pattern.
2. More than one observer. You, your spouse, the teacher, the case manager, the child themselves if they are old enough. Different people see different things, and that is the point.

MODULE 2

Naming the Priority Problem

Two assessments give you a pattern. The pattern gives you one sentence the whole support team can act on.

“When [this reminder happens], in [this setting], [child] responds by [this behavior], and what helps is [this].”

WORKED EXAMPLE

“When John is told he is out of time on a task, at school or at home, he responds by swearing and leaving the room, and sometimes by pushing past whoever is nearest. What helps is a five-minute warning, a named stopping point, and a way to finish later.”

Notice what that sentence does: it names a trigger, a setting, a behavior and a strategy. Every person on the team can now do the same thing.

WHAT MAKES IT A GOOD ONE

- ONE problem, not a list. Pick the one that causes the most harm or happens most often.
- Built from two assessments, not one incident.
- Written in observable behavior, not interpretation.
- Includes what already helps, even a little.
- Agreed by the team, so everyone responds the same way.
- Revisited. When it stops being the priority, write a new one.

MODULE 2

Predictability and Consistency

The two changes that reduce triggers more than anything else you can do.

PREDICTABILITY

A child who knows what is coming does not have to scan for danger. Visual schedules, the same order of operations every morning, naming what happens next before it happens, warning before transitions.

CONSISTENCY

- Can children count on what you say and do?
- Are you balanced and fair, on a bad day too?
- Are your kids free from worrying about the unknown?

> ***"Success is the sum of small efforts, repeated day in and day out."***
>
> Robert Collier

The hardest part of being consistent is keeping your own responses calm. With children who have experienced trauma, your ability to stay calm is one of the single best things you can do to help a child stay calm or get calm. We are working toward progress, not perfection.

MODULE 3

The Managing Emotions Guide (MEG)

You build it WITH the child. It is the single most useful document in the model.

STEP 1 • UNDERSTANDING MY EMOTIONS

Across all four stages, the child answers:
What flips my switch? • What am I thinking or paying attention to? • What do I feel, and what does my face and body show? • What am I doing or wanting to do?

STEP 2 • MANAGING MY EMOTIONS

For each stage, two lists:
Things I can do. • Things an adult or a friend can help me with.

That second list is where you find out what the child actually wants from you.

1

Use the child's own words

If she says “I start freaking out,” write that. Then later you can say, “I notice you are starting to freak out. What can I do to help?”

2

Drawing counts

Some children would rather draw it or describe it while you write. Both are fine.

3

Share it, with permission

Grandparents, teachers, coaches, the school counselor, the case manager. Anyone who can help.

4

It is a living document

Update it. Children change. So do their triggers and what works.

MODULE 3

Emotional Hotspots

Specific times of day that reliably set a child off. Every household has them. Name yours and you can plan around them.

Bedtime

Dark, alone, unsupervised. Historically the least safe hours.

Mornings

Rushed, loud, full of directions and time pressure.

Transitions

Stopping one thing and starting another. Loss of control, twice.

Homework

Failure is visible. So is the adult watching.

Meals

Food scarcity leaves a long memory. So does what happened at the table.

Bath or shower

Undressed and unguarded. A hotspot for many children, for obvious reasons.

In public

No exit, unpredictable strangers, and an audience for any correction.

Vacations and holidays

Every routine gone at once, plus anniversary reminders.

WHY THESE, SPECIFICALLY

Almost every hotspot involves either a transition or a loss of control, and often both. Children who have experienced trauma frequently have a low tolerance for frustration, and the mere word “no” can feel threatening. Once you know your household's hotspots, you can pre-teach into them, add predictability around them, and stop being surprised by them.